Ex:

- P - Risk for trauma related to dizziness.
- I - Instructed to call for assistance when getting Out of Bed. Put the Call light in Reach.
- E - Consistently call for assistance before getting Out of Bed because the patient continues to experience dizziness.

It follows the nursing process and uses nursing diagnoses while placing the plan of care within the nurses' progress notes.

9.4.3 Focus Charting

Focused only on nursing diagnosis, patient problem, signs, symptom, or event and writing the interventions it has three components (DAR)

- DATA – subjective or objective that supports the focus (concern)
- ACTION – nursing intervention
- RESPONSE – Patient response to intervention

Ex:

- D – complaining of pain at incision site, pain score: 7/10
- A – Repositioned for comfort. -------Analgesics 50mg IM given.
- R – States a Decrease in Pain, "Feels Much Better."

9.4.4 Charting by Exception

A longhand note is written only when the standardized statement on the form is not met

Ex.

Mr. Sunil, 70 yrs is going to the doctors room and finding out what doctors are doing with his chart and report. Usually this type of behaviour is not mentioned in the chart. But out of interest we can go for mentioning in separate chart to highlight his anxiety behaviour

- Highlights abnormal data of patient
- Decreases narrative charting time
- Eliminates duplication of charting.

9.4.5 Computer-Assisted Charting

- It is an Electronic Health Record (EHR). Computerized record of patient's history and care across all facilities and admissions.
- It initiates Computerized Pprovider Order Entry (CPOE) which saves time.
- Provides efficient work flow.
- Documentation done as interventions are performed using bedside computers.
- Some produce flow sheets with nursing interventions and expected outcomes.
- Others use a POMR format to produce a prioritized problem list.



Advantages

- Date and time of the notation automatically recorded.
- Notes always legible and easy to read.
- Quick communication among departments about patient needs.
- Many providers have access to patient's information at one time.
- Can reduce documentation time.

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